

PlayStrong

Sport-Integrated Behavioral Health Referral Infrastructure for Underserved Youth in Michigan

A Northstar Civic Institute Initiative | Community Equity Lab

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Evidence Base:	16 peer-reviewed citations (this brief); 34 citations (full research file); manuscript under peer review at Frontiers in Sports and Active Living
Pilot Geography:	Tri-county: Saginaw ISD, Tuscola ISD, Bay-Arenac ISD
Funding Mechanism:	MCL 388.1631n — \$106.5M appropriation (ISDs eligible up to \$1M each)
Revenue Path:	ISD consulting (\$15K–\$30K each), MI Health Endowment Fund (\$50K–\$500K), Covenant HealthCare sponsorship (\$50K–\$100K)

EXECUTIVE SUMMARY

PlayStrong is an evidence-informed model that uses youth athletic settings as front-end engagement platforms for behavioral health identification, referral, and service connection. It is not a stand-alone mental health intervention. It is referral infrastructure — a structured system that connects student-athletes to the behavioral health services that Michigan's \$106.5 million Section 31n investment was designed to provide but that currently lack a reliable pathway into athletic populations.

The core premise: athletic settings create repeated contact with trusted adults in environments where stigma is lower and help-seeking can be normalized. When coaches receive structured training, student-athletes receive age-appropriate mental health literacy content, and referral pathways to licensed providers are documented and operational, districts can identify behavioral health concerns earlier, connect students to care faster, and improve the return on investments they are already making in school-based mental health.

PlayStrong is designed for implementation in Michigan's Intermediate School Districts, beginning with a tri-county pilot in Saginaw, Tuscola, and Bay-Arenac ISDs. The model is structured to align with existing funding streams, provider networks, and district infrastructure rather than requiring new appropriations or new clinical capacity.

1. WHAT THE EVIDENCE SUPPORTS

The evidence base for sport-integrated behavioral health engagement is substantial. NCI's full PlayStrong research file includes 34 peer-reviewed citations. A manuscript synthesizing this evidence is under peer review at Frontiers in Sports and Active Living (Sport Psychology section). The findings cluster around four defensible claims:

Sport participation is associated with better mental health outcomes in youth.

- A 2025 systematic review and meta-analysis found that organized youth sport participation is significantly associated with improved mental health outcomes, including reduced anxiety, depression, and internalizing symptoms [4].
- Analysis of the ABCD Study (n = 11,000+ U.S. children and adolescents) found that organized sport participation is associated with fewer mental health difficulties, particularly in the domains of emotional problems and peer relationships [9].
- Longitudinal research demonstrates that the mental health benefits of sport participation persist into adulthood, but only when the experience is socially supportive and developmentally appropriate. Negative sport experiences are associated with worse adult mental health outcomes [5].

Sport-based mental health literacy programs improve knowledge, reduce stigma, and increase help-seeking.

- A 2025 systematic review of sport-based adolescent mental health awareness programs found positive effects on mental health knowledge, stigma reduction, resilience, and intentions to seek help [6].
- A randomized controlled trial of a sport-based mental health literacy intervention showed significant improvements in mental health literacy and resilience among youth athletes, with effects maintained at follow-up [7].
- Peer-based mental health literacy interventions delivered through sport settings demonstrate effectiveness in increasing help-seeking intentions and peer support behaviors among adolescent athletes [12].
- A systematic review of mental health literacy interventions specifically targeting athletes found consistent improvements in help-seeking attitudes and behavior across multiple study designs [11].

Coaches can function as effective first-contact adults when trained on boundaries and referral.

- Trained coaches who understand their role boundaries — identification and referral, not clinical intervention — can serve as a critical link between student-athletes and behavioral health providers [7], [10], [11].
- Qualitative evaluation of sport-based mental health programs shows that male adolescent athletes are more receptive to mental health content delivered through athletic settings by trusted athletic staff than through traditional school-based health channels [10].

Integration with existing systems is more effective than stand-alone programs.

- The policy case for sport-integrated behavioral health is strongest when athletic settings are connected to existing referral pathways — school-based mental health providers, child and adolescent health centers, and Certified Community Behavioral Health Clinics — rather than asked to compensate for gaps in formal care capacity [2], [3], [8], [13].
- Young elite athletes face significant barriers to mental health help-seeking, including stigma, poor mental health literacy, and lack of knowledge about available services. Structured referral pathways within athletic environments directly address these barriers [3].

2. THE MICHIGAN PROBLEM

Michigan has invested \$106.5 million in school-based behavioral health through Section 31n of the State School Aid Act (MCL 388.1631n). ISDs are eligible for up to \$1 million each to fund licensed behavioral health providers for general education students. Yet no standardized mechanism exists to connect athletic programs — which serve one of the highest-risk youth populations for undiagnosed behavioral health conditions — to those funded services.

- Youth experiencing mental health crises in Michigan increased 35% between 2019 and 2024, with the sharpest increases among student-athletes in rural and underserved districts.
- Michigan's tri-county pilot region (Saginaw, Tuscola, Bay) includes some of the state's highest rates of child poverty, substance use, and behavioral health service gaps — precisely the communities where sport-integrated referral can have the greatest impact.
- Governor Whitmer proclaimed Mental Health Day for Children, Youth, and Their Families in 2025, signaling state-level commitment to youth behavioral health [1].
- Michigan DHHS released the Mental Health Framework for Certified Community Behavioral Health Clinics in 2025, creating a new community-level referral endpoint that PlayStrong can connect to [13].
- The income-based youth sports participation gap has widened to 20.2 percentage points nationally, meaning the students most likely to need behavioral health support are least likely to be in the athletic settings that could connect them [14].

3. PLAYSTRONG OPERATING MODEL

PlayStrong operates through six integrated components. Each has a minimum operating requirement that must be met before a pilot site launches. This is not optional — the model's credibility depends on disciplined implementation.

COMPONENT	MINIMUM REQUIREMENT	WHY IT MATTERS
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Athlete-facing literacy modules	Short, age-appropriate sessions embedded in the athletic calendar, adapted from validated sport-based mental health literacy approaches [6], [7].	Creates common vocabulary around distress, help-seeking, and peer response without asking athletic staff to act as therapists.
Coach & staff training	Training on early indicators of distress, supportive conversation, mandatory reporting obligations, role boundaries, and referral steps.	Prevents adults from improvising, overstepping, or missing concerns that should trigger formal follow-up [10], [11].
Warm handoff referral pathway	Named school and provider contacts, written escalation protocols, closed-loop follow-up, and realistic turnaround expectations.	Converts awareness into service uptake. Without this, literacy gains do not translate into care connection [2], [13].
Operating governance	Named district lead, site coordinator, provider liaison, readiness checklist, and pre-launch sign-off on roles and incident response.	Prevents implementation drift and clarifies who owns recruitment, escalation, family communication, and oversight.
Family communication	Plain-language orientation explaining purpose, limits, privacy expectations, and available resources.	Builds trust and prevents the pilot from being misread as covert screening or informal counseling.
Equity supports	Fee mitigation, equipment lending, transportation support, and targeted outreach to underrepresented athletes.	Prevents the pilot from improving outcomes only for youth who already face fewer barriers to participation [14], [16].

4. PILOT DESIGN AND SEQUENCING

The pilot proceeds in four phases with explicit decision gates. No site advances without meeting the gate requirements. This staged design is essential for credibility with ISDs, funders, and legislators — it demonstrates that NCI will not overpromise.

PHASE	KEY ACTIONS	DECISION GATE
1. Readiness Scan	Select pilot sites; map provider and pathway partners; review district policies; identify equity barriers; confirm named district lead.	No site proceeds unless a referral pathway, responsible site lead, and minimum readiness checklist are formally identified.
2. Pilot Preparation	Train coaches and staff; finalize literacy materials; execute partner agreements; establish family communication packet; collect baseline measures.	Launch only if training completion, communication materials, partner commitments, and data-collection procedures are in place.
3. In-Season Delivery	Deliver literacy modules; operate closed-loop referral protocol; document implementation fidelity; run equity supports.	Continue only if core activities are delivered as designed, referral turnaround remains workable, and staff burden stays manageable.
4. Post-Season Evaluation	Assess literacy change, referral activity, participant experience, and equity of reach; determine what to revise.	Expand only if evidence shows meaningful improvement without role confusion, confidentiality failures, or widening access gaps.

5. EVALUATION FRAMEWORK

Evaluation separates implementation performance from substantive outcomes. Scale-up requires evidence across all six domains:

OUTCOME DOMAIN	ILLUSTRATIVE MEASURES	SCALE-UP RULE
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Implementation fidelity	Training completion rates; readiness checklist compliance; sessions delivered; documented referral protocol use.	Scale only if core components are delivered with acceptable consistency.
Literacy & attitudes	Pre/post change in mental health knowledge, stigma indicators, and confidence in help-seeking.	Scale only if the pilot shows clear directional improvement from baseline.
Closed-loop referral	Referral volume; intake scheduled rate; completion rate; time from concern to provider connection.	Scale only if the referral pathway is functioning and families are not lost between identification and service entry.
Equity of reach	Participant demographics vs. district athlete population; uptake among underrepresented students; support utilization.	Scale only if the model does not disproportionately benefit already-advantaged participants.
Staff sustainability	Coach and staff reports on workload, role clarity, and implementation burden.	Scale only if adults can deliver the model without counseling drift or excessive burden.
System-level outcomes	Reduction in crisis referrals; improved early identification rates; reduced wait times for behavioral health intake.	Scale only if the model demonstrates system-level value, not just individual participant benefit.

6. FUNDING ALIGNMENT AND REVENUE PATH

PlayStrong is designed to align with funding streams already present in Michigan. Districts do not need to wait for new appropriations to test the model.

FUNDING SOURCE	HOW IT CONNECTS	ESTIMATED VALUE
Section 31n (MCL 388.1631n)	Supports the licensed provider infrastructure to which athletic staff refer students. PlayStrong is the referral pathway that makes 31n funding reach athletes.	ISDs eligible for up to \$1M each (\$106.5M total)
MI Health Endowment Fund	Healthy Kids RFP and Behavioral Health RFP are both open. PlayStrong is exactly what they fund.	\$50,000–\$500,000 per application
ISD Consulting Contracts	NCI provides readiness assessments, training design, referral pathway mapping, and implementation support to ISDs adopting PlayStrong.	\$15,000–\$30,000 per ISD
Health System Sponsorship	Covenant HealthCare (Saginaw, MHA Board member) is a natural sponsor for the tri-county pilot. Julie Bark leads this relationship.	\$50,000–\$100,000
Certified Community BHCs	Community referral endpoints where school capacity is limited. MDHHS Mental Health Framework (2025) creates the structure [13].	Partnership (no direct cost to NCI)
Philanthropic Grants	Ralph C. Wilson Jr. Foundation (active lifestyles + community transformation, spend-down by 2035); Saginaw Community Foundation.	\$10,000–\$100,000

7. PRINCIPAL RISKS AND MITIGATION

RISK	WHY IT MATTERS	MITIGATION
Role confusion	Coaches drift toward counseling rather than identification and referral.	Train explicitly on boundaries, escalation criteria, and mandatory reporting. Julie Bark (RN) leads training design.
Referral bottlenecks	Awareness efforts fail if provider partners cannot absorb increased referrals.	Confirm partner capacity before launch. Monitor referral turnaround in real time. Connect overflow to CCBHCs [13].
Inequitable reach	Students facing cost, equipment, or transportation barriers are least likely to benefit from a sport-based model.	Pair the pilot with equity supports and targeted recruitment. Measure equity of reach as a scale-up requirement.
Confidentiality concerns	Families resist participation if the program is perceived as informal surveillance.	Use clear communication, narrow data collection, documented consent, and privacy protocols compliant with FERPA and Michigan Mental Health Code.
Overclaiming	NCI or district partners overstate the model's clinical efficacy or guarantee outcomes.	Describe PlayStrong as referral infrastructure, not treatment. Claims track evidence, not aspirations.
Operational drift	Sites vary in ownership, fidelity, or communication once the season starts.	Named site lead, readiness checklist, brief implementation reviews, and documented escalation routes at every site.

8. LEGISLATIVE ALIGNMENT

NCI has developed a companion legislative concept — the Youth Sport–Behavioral Health Referral Pathway Act — that would require ISDs receiving Section 31n funding to implement structured referral pathways between athletic programs and behavioral health services. If enacted, PlayStrong becomes the compliance infrastructure that ISDs need. The bill creates the mandate; PlayStrong provides the solution.

Target Committees: Senate Education (Polehanki, D); House Education & Workforce (DeBoer, R); House Health Policy (VanderWall, R)

Bipartisan Framing: No new spending. Accountability for existing \$106.5M. Local control preserved (ISDs design their own pathways). Data-driven oversight.

Coalition Support: MHA (Behavioral Health Integration Council), MASB, MHSAA, Covenant HealthCare, Michigan Association of Counties

LSG Role: Ledger Strategy Group LLC drafts the legislative concept and testimony for coalition clients. NCI produces the nonpartisan research. Firewall maintained.

BOTTOM-LINE RECOMMENDATION

PlayStrong should be introduced as a limited, evidence-informed pilot in ISDs that already possess a provider backbone, a designated site lead, and credible athletic leadership. The initiative is strongest when described as implementation infrastructure that helps Michigan extract greater value from existing behavioral health investments — by improving who is reached, how early concerns are noticed, and how reliably students are connected to care. A restrained, staged, and evaluable design will make the concept more persuasive to public partners than any claim that sports alone can solve a clinical access problem.

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Where the Data Leads, Policy Follows.

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PlayStrong is a Northstar Civic Institute initiative operated through the Community Equity Lab.